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Proposal for Social and Non Governmental Organization [NGO] Assessment for National
Acquired Immune Deficiency Syndrome [AIDS] Control Project 2 - Proposal

**Proposal for Social & NGO Assessment
for
National AIDS Control Project II**

PROJID 768184



Submitted by



**Social Services Area
Administrative Staff College of India
Bella Vista, Hyderabad - 500 082.**



G217927

Proposal for Social and NGO Assessment for

National AIDS Control Project II

Introduction:

South East Asia is currently passing through a major HIV/AIDS Epidemic. Since the first AIDS case was detected in Bombay in 1986, a total of 2512 cases have been reported in the country till Feb 1996. By now this number must have crossed 3000. Due to covert and insidious nature of HIV, the above numbers reflect only the tip of the iceberg. It is well known that a multitude of social, economic and cultural factors determines the spread of HIV. While states like Maharashtra, Tamil Nadu and Manipur have a higher prevalence of the problem, it is seen in all states and union territories of the country.

The National sero-surveillance system, started in 1987, was revamped in 1992 with the support of World Bank loan. The overall objective was to slow the spread of HIV in the country and thereby reduce future morbidity, mortality and economic impact of AIDS. The specific objectives of the programme are to:

- a. enhance public awareness on HIV transmission and prevention
- b. ensure proper screening of blood/blood products and decrease the practice of professional blood donation
- c. strengthen STD control
- d. develop skills in counselling, health education and clinical management
- e. develop and implement interventions aimed at groups known for high risk behaviour
- f. institute sentinel surveillance covering clients attending STD clinics, intravenous drug users and women attending antenatal clinics
- g. involve all states and union territories to develop their capabilities to control HIV

In each state, attempts would be made to identify the NGOs involved in health/women's welfare/community mobilisation activities in social sector. From the identified NGOs, four would be sampled in such a way that two (one big and one small) involved in HIV/AIDS activities and two (one big and one small) not involved in HIV/AIDS activities so far to undertake institutional analysis. A detailed needs assessment would be undertaken on the selected NGOs with the help of a check list to ascertain their capacities and identify the potential areas for collaboration with public sector in order to improve the access and utilisation of safe sex practices. In each of selected state a detailed case study of one NGO which used innovative approaches to contain the spread of HIV would be undertaken.

In addition, discussions will be held with bilateral and multi-lateral donor agencies to know their involvement in HIV support and financial commitment.

Community perceptions:

Efforts will be made to obtain the community perceptions by conducting focus group discussions both in urban and rural areas in each state. General community perceptions regarding illness, health-seeking behaviour, reproductive and child health, preferred systems of medical care, gender discriminations in health seeking etc. will be elicited in rural areas. Specific focus will be given in the urban areas on awareness regarding HIV, sexuality and safe sex practices adopted by the high-risk groups.

In each of the eight selected states, five focus groups will be held in urban areas covering commercial sex workers, uniformed forces, taxi/auto drivers, long distance truck drivers and college students. The focus groups discussion in rural areas will be carried out in the two selected PHCs in each state. In each PHC area two sub centres will be selected. Four focus groups will be conducted (two for men and two for women) covering the main village as well as the hamlet inhabited by weaker sections following a circular systematic procedure which ensures a representation of all social groups.

In each state ten in-depth case studies will be undertaken covering two commercial sex workers, two sero- positives, four persons suffering from STDs and two known IV drug users. The focus of the case studies would be on awareness of the causes of HIV/AIDS and its treatment, accessibility of health services, attitude and behaviour of

High risk groups:

CSWs	150 (ten from each town/city)
Known intravenous drug user	16 (two from each state)
Police	120 (fifteen from each state)
Taxi/auto drivers	120 (fifteen from each state)
Traditional healers from tribal areas	10

Observational studies:

The proposed tools include non-participatory observation of interactions between clients utilising the AIDS control services and providers. These include STD clinics attached to medical colleges located at the sampled sites, HIV surveillance centres and certified blood banks.

Assessment of institutional capacity:

According to the recent directory of NGOs published by Central Social Welfare Board, there are nearly six thousand NGOs operating in the states selected.

State-wise distribution of NGOs*	
State	Number
Andhra Pradesh	669
Tamil Nadu	624
Maharashtra	1169
Goa	35
Rajasthan	285
Delhi	126
UP	655
West Bengal	725
North East	1017
Total	5747
* Source: Directory of NGOs, Central Social Welfare Board 1995-96	

The study proposes to use rapid appraisal techniques to facilitate qualitative data collection. The ASCI will identify collaborating institutions in different parts of the country. In order to ensure unbiased data collection, preference will be given to those institutions/research groups which were not directly involved in the implementation of AIDS project I. There will be an initial workshop in which representatives of the participating agencies, NACO and the donor agencies would participate to discuss and finalise the protocol and study instruments.

Coverage and Study tools:

Interviews:

Semi structured interviews using pre-tested formats will be undertaken in the selected areas covering the community, high-risk groups, service providers and programme managers. While developing the instruments due attention will be given to social issues, high-risk behaviours, service delivery constraints, innovations attempted and experiences.

Program managers:

National	3
State and District levels	24 (3 from each state)
Officers in-charge of surveillance centres	8
Officers certifying blood banks	8

Service providers in primary, secondary and tertiary levels

Public:

Professors of Medical Colleges	24 (STD, Obst, TB/Inf. diseases)
Specialists at district/sub district	16 (Medical & Obstetrics)
PHC medical officers	32 (16 each from high & low)
Multipurpose Health Workers & Anganwadi Workers	64 (32 each from high & low)

Private:

Medical Practitioners	40 (5 from each state)
IMA representatives	16 (2 from each state)

The desk review:

This would have specific focus on current knowledge regarding the prevalence and spread of HIV/AIDS in India. Based on the review it would attempt to describe the nature and characteristics of "high risk" populations. It would also include a Delphi study of the state program managers involved in prevention and control of AIDS. A literature search would also be undertaken to highlight the innovations and experiences from the first National AIDS Control Project. These two reviews would focus on identifying the social and gender barriers in creating AIDS awareness, especially in the high-risk groups, and bring about social and attitudinal changes.

In-depth study:

Based on the analysis of preliminary data the in-depth study would be restricted to limited sites identified as "high risk" areas in the country. The National surveillance data being collected by 62 centres operating in different parts of the country reported that 22,389 samples out of the 28 lakh samples screened, up-to Feb 1996, as positive for HIV. One significant finding was that HIV infection is present in all states and territories. Despite the fact that this screening is restricted to special groups known to be carrying high risk of HIV, it nevertheless identified states in advanced stage of AIDS epidemic which includes Maharashtra, Tamil Nadu and Manipur. There are several other states where the epidemic is in an early phase. Considering the time constraint it is proposed to cover the states and localities, which are known "hot spots" of the problem and cover other states giving a spatial distribution. In addition, some special areas inhabited by marginalized sections of the society such as tribals would also be included. The suggested states/localities include:

State	Location
Andhra Pradesh	Hyderabad, Tirupati, Adilabad
Tamil Nadu	Chennai, Madurai
Maharashtra	Mumbai, Pune, Aurangabad
Rajasthan	Jaipur, Jodhpur
Punjab	Chandigarh, Ludhiana
UP	Varanasi, Kanpur
West Bengal	Calcutta, Darjiling
North East	Manipur

The GOI will be completing phase I of this project by 1999 and is currently in the process of negotiation with the Bank for AIDS project II. At this juncture, it is appropriate to undertake a detailed stakeholder and institutional analysis to facilitate comprehensive understanding of the problem in the Indian context. This helps to address the key issues and develop better strategies to contain and reduce the burden of HIV/AIDS in India during the next five years under the next phase of the project.

Objectives:

General:

To provide a better understanding of the behaviours, experiences and impressions of the key stakeholders and beneficiaries of the project.

Specific:

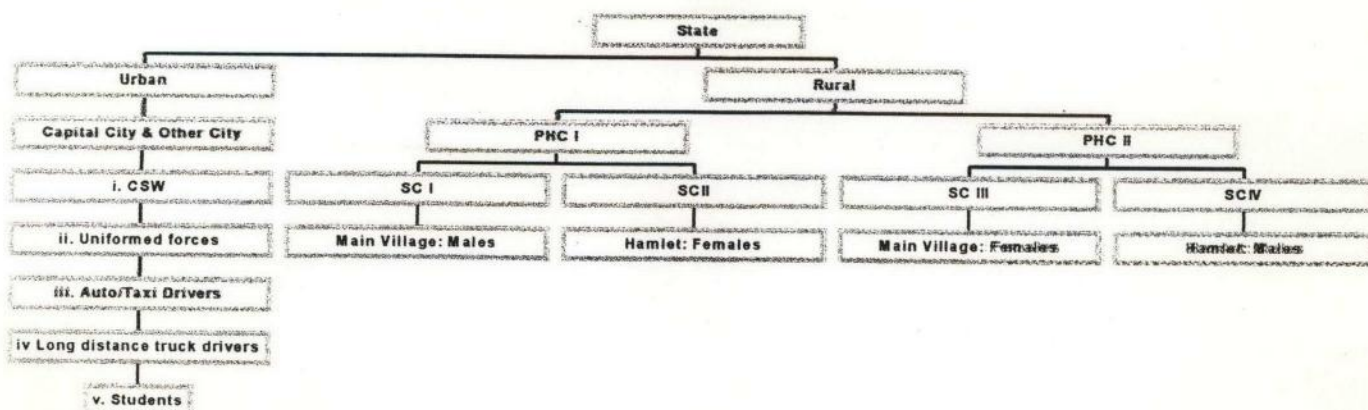
1. To suggest strategies that help in improving the reach of services to "high risk but hard to reach" groups such as commercial sex workers, localised intravenous drug users, mobile and itinerant workers such as truck drivers.
2. To what extent there is a significant and sustained behavioural change in-terms of safe sexual practices among the high risk groups
3. To highlight the aspects which make females more vulnerable to HIV/AIDS and suggest a proactive gender development strategy
4. To emphasise the need for flexible approaches in devolving service delivery mechanisms taking into account the local social, economic and cultural conditions.
5. An institutional analysis of capabilities in terms of strengths and weaknesses of NGOs and other community based groups such as women, youth, panchayat raj etc., in promoting community awareness and offer health care.

Approach:

The proposed social assessment would have two components. The first would be a desk review of existing situation using secondary data. For the second component of the study, in-depth qualitative information would be collected from different locations in the country sampled on the basis of desk review.

health staff, costs, and local beliefs and practices in the community. Thus, a total of 100 case studies are proposed.

Assessment of Community Perceptions Study Design



Outputs:

Activity	Group	Coverage	Expected outcome
Structured interviews	Program managers:	43	Provider's perceptions
	Public service providers	136	
	Private providers & representatives of professional bodies	56	
	High risk group	416	Client perceptions
	NGO representatives	32	Assessment of Institutional capacity
Observational studies	Bilateral & multilateral agencies	5	Future opportunities & collaborations
	STD clinics	16	Provider behaviour, privacy, Client provider interactions, communication
	HIV surveillance centres	16	
	Blood Banks	16	
	Urban	50	Community perceptions regarding illness, health seeking behaviour, preferred systems of health care, gender discriminations in health seeking
Case studies	Rural	32	
	CSWs, Sero +ves, Persons suffering from STD and IV drug users	100	

Time frame:

Activity	March	April	May	June	July
Desk Review					
Net working with collaborating institutions					
Initial Workshop					
Finalisation of study protocol and tools					
Training of field investigators					
Data collection					
Data compilation and analysis					
Draft report and workshop					
Final report					

Budget:

Institution	Head	Rs.
ASCI	Professional fee	600000
	Computer services for data entry and analysis	150000
	Travel for study co-ordination team	200000
	Work shops (Two)	600000
	Printing & stationary	150000
	Institutional support	100000
	Incidentals	30000
	Sub Total	1830000
Collaborating Institutions	Training, data collection & scrutiny- 3 months	1296000
TOTAL		3126000

Data Collection:

<i>Estimates for one state</i>	<i>Rs. 1,62,000 for 3 months</i>
<i>One co-ordinator</i>	<i>@ Rs. 25,000 per month</i>
<i>One senior investigator</i>	<i>@ Rs. 8,000 per month</i>
<i>Two field investigators</i>	<i>@ Rs. 3,000 per month</i>
<i>Travel & per diem</i>	<i>@ Rs. 12,000 per month</i>
<i>Institutional support</i>	<i>@ Rs. 3,000 per month</i>
<i>Incidentals</i>	<i>@ Rs. 3,000 per month</i>

Costing of Training Component: II HIV/AIDS Control Project

Terms for Reference for Prj DB Gupta

1. Mr. DB Gupta will assist and facilitate the National AIDS Control Organisation, MOHFW in the process of preparing the proposed II HIV/AIDS Control Project.
2. Specifically Mr. DB Gupta will under the guidance of Mr. Prasada Rao, Additional Secretary, and Director, NACO, MOHFW.
 - 2.1. Provide support to the NACO Project Preparation Team in preparing training costs.
 - 2.2. Prepare the Project Implementation Plan (PIP) on the basis of the discussions of the Project Preparation Team.
 - 2.3. Revise the training costs and the PIP for the training component as the basis of discussions and agreements with NACO.
3. Schedule
 - 3.1. A draft project Implementation Plan for the training component and the training costs be made available by May 5, 1998
 - 3.2. A revised PIP (Training Component) and Training Costs be prepared and made available on dates to be mutually agreed upon.
 - 3.3. The final training costs/PIP for training component should be made available by May 15, 1998.

Second HIV/AIDS Control Project

TOR for Mr. Prof D.B. Gupta, NCEAR, Delhi

Short study to identify and measure costs in diagnosing and treatment of HIV & TB at selected hospitals.

1. Mr. D.B. Gupta will undertake the study of identifying and measure costs of diagnosing and treatment of HIV and TB at these hospitals currently handling HIV patients. The hospitals currently identified are located in Mumbai, Chennai and Manipur.
2. Specifically Mr. D.B. Gupta will identify and measure all direct and indirect costs of inpatient, and outpatient; diagnosis and treatment.
 - 2.1. For this purpose he will undertake visits to the selected hospitals to hold consultations with the staff on the cases handled by the hospitals, and the nature of hospital records kept by them. He will on the basis of these discussions make an assessment of the Socio-economic background of the inpatients and outpatients.
 - 2.2. The consultant will then prepare proforma for the hospitals and a short questionnaire for a survey, for the inpatients/outpatients on treatment/diagnosis costs and for working out the indirect costs.
 - 2.3. The consultant will collate and analyse the data to workout the unit cost for each type.
 - 2.4. The consultant will be provided such research and field support as may be necessary in the conduct of the study.
3. The draft report on unit costs will be submitted by May 30, 1998.
4. **Proposed Budget**
 - 4.1. In addition to consultancy costs, other costs will be the provision of research and secretarial assistance, computer processing, travel and communication costs.